in goals of care notes identified as negative by community members.

2. Upon successful completion, participants will be able to discuss reasons why phrases commonly identified in negative notes may be associated with negative perceptions by community members.

Key Message: Specific phrases, such as descriptions of patient or surrogate requests for life-prolonging treatment, were more often identified in documented goals of care conversation (GOCC) notes perceived as negative by community members. Future research should explore patient and clinician predictors of the use of these phrases in documented GOCC notes.

Abstract

Background/Rationale. Goals of care conversations (GOCC) among clinicians, patients and surrogates aim to clarify and align patient values and preferences for medical care with treatment options (1-3). Qualitative data suggests that community members perceive that negative content (e.g. lack of empathy) exists in GOCC notes documented in the electronic healthcare record (EHR) (4). This negative content, which can be read by clinicians and patients, has the potential to harm clinician-patient relationships and exacerbate racial/ethnic disparities in patient care (5).

Objectives/Purpose. To identify the most common phrases associated with documented GOCC notes perceived as negative by community members.

Methods/Approach. This multi-center study assessed community member perceptions of EHR documented GOCC notes written by clinicians across 14 hospitals. Each documented GOCC was reviewed by three different community members. Reviewers rated each note as negative, neutral or positive based on their perceptions of the overall content of each note. We used Term Frequency-Inverse Document Frequency (TF-IDF) analyses to identify three word phrases most commonly associated with negatively perceived GOCC notes.

Results. Of 966 documented GOCC, 13% were perceived as negative. Three types of phrases were commonly identified in these notes. First, phrases referring to palliative consults, such as "see palliative note" or "see palliative care". Second, phrases describing patient or surrogate preferences for life-sustaining treatment, such as "remain full code", "want tube feed" and "cancer directed treatment." Third, phrases discussing poor prognosis, such as "discuss poor prognosis" and "treatment prognosis poor".

Conclusion/Implications. Specific phrases, such as descriptions of patient or surrogate requests for life-prolonging treatment, were more often identified in documented GOCC notes perceived as negative by community members. Future research should explore patient and clinician predictors of the use of these phrases in documented GOCC notes and assess whether use of these phrases may affect clinician-patient relationships and patient outcomes.

Keywords. Pain management, Symptom management, Opioids, Onoclogy, Psychosocial interventions, Perioperative.

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Community-based palliative support: Rural Community Health Workers Transform Care Delivery

Erin Collins, MN RN CHPN, The Peaceful Presence Project. Oralia Mendez, MPH CHW, Oregon State University Extension Service - Family and Community Health. Sandi Phibbs, PhD MPH, Oregon State University. Seiko Izumi, PhD RN FPCN, Oregon Health & Science University School of Nursing.

Outcomes:

1. Upon successful completion, participants will be able to explain how a community health worker can provide palliative care in a rural community.

2. Upon successful completion, participants will be able to summarize three key takeaways of learners in a palliative care continuing education course for community health workers.

Key Message: Training Community Health Workers in primary palliative care and incorporating the role into community-based teams is an effective strategy to transform the reach of palliative care in rural communities.

Abstract

In Oregon, 26 of 36 counties are designated rural or frontier areas. In these areas, access to healthcare for an advanced illness often means traveling hundreds of miles. Access to specialty palliative care in rural areas in Oregon is virtually non-existent. Community health workers (CHWs) are non-clinical frontline health workers who reside in the community and have the potential to fill the gap in palliative care needs by providing local support. In Oregon, 20 hours of continuing education is required every 3 years to maintain registration with the Oregon Health Authority. We developed a Palliative Care Continuing Education course for CHWs to empower them to deliver primary palliative support to the people they serve in rural Oregon. We trained a total of 40 CHWs working in diverse practices including rural and federally qualified health centers and community-based organizations. The hybrid online 12-hour course was delivered over 4 weeks using didactic and interactive role-play components. The curriculum included principles of palliative care, advance care planning, spiritual health and caregiver assessment and communication skills. We evaluated the impact of the training through pre-and post-training surveys and semi-structured interviews 60 days after the training. CHWs demonstrated a significant increase ($p< 0.001$) in knowledge of palliative care, confidence in advance care planning and assessing total pain, and supporting patients and family caregivers. In the interviews, participants described how they use the skills learned to educate patients in practical symptom management and completing advance directives and recognizing and supporting caregivers who are experiencing grief. This education program was effective at preparing CHWs to deliver primary palliative care in rural communities where access to healthcare, particularly specialty palliative care, is scarce. Integration of CHWs as valuable members of a community-based palliative care team would be a strategy to transform palliative care reach in rural communities.

Keywords. Pain management, Symptom management, Geriatrics, Onoclogy, Psychosocial interventions, Quality improvement, Hospice, Perioperative.

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Comparing ChatGPT and DeepSeek for Deprescription Support in Geriatric and Palliative Care: An Exploratory Study

Gokhan Demir, MD, HCA Aventura Hospital. Sneha Akurati, N/A, University Of Miami Miller School of Medicine. Angel Sánchez, N/A, HCA Aventura Hospital. Sanaz Kashan, MD, Catholic Hospice, HCA Aventura Hospital.

Outcomes:

1. Upon successful completion, participants will be able to evaluate the strengths and limitations of ChatGPT and DeepSeek in supporting deprescription decisions for older adults with polypharmacy.

2. Upon successful completion, participants will be able to apply principles of AI-assisted decision support to critically assess deprescription opportunities in geriatric and palliative care practice.

Key Message: Polypharmacy will remain a major issue in geriatrics and palliative medicine, as it is associated with falls, adverse drug events, and hospitalizations. Deprescription will continue to be underutilized due to limited time, fragmented guidelines. AI language models such as ChatGPT and DeepSeek may offer decision-support that will improve deprescription efficiency.

Abstract

Background. Polypharmacy is common in older adults and increases risks of adverse drug events, falls, and hospitalizations. Deprescription can mitigate these harms but is often hindered by time constraints, fragmented guidelines, and complex comorbidities. Artificial intelligence (AI) large language models (LLMs) may provide decision-support for deprescription in geriatric and palliative care.

Objective. To compare ChatGPT and DeepSeek in their deprescription recommendations across standardized polypharmacy vignettes.

Methods. Three validated clinical vignettes, varied by activities of daily living (ADL) status and cardiovascular disease (CVD) history, were presented to ChatGPT (GPT-4/5) and DeepSeek (2024 version). Each vignette was tested in three replicates per model. Outputs were analyzed for (1) binary deprescription decisions, (2) medications targeted, and (3) replicate consistency.

Results. In patients without CVD, both models consistently recommended deprescription across all ADL